Studying Development of Psychopathology Across the Lifespan

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Abstract

The field has not adequately studied individuals' development of psychopathology across the lifespan, instead taking a piecewise approach to development, limiting our understanding of lifespan development and the origins of psychopathology. We describe why this is the case—beyond logistical challenges. First, diagnostic and taxonomic systems do not define psychopathology constructs from a developmental perspective. A developmental perspective is crucial because psychopathology constructs often have earlier origins than their full clinical presentation, and forms of psychopathology frequently show changes in behavioral manifestation across development (heterotypic continuity). Second, current assessments do not adequately capture these changes in behavioral manifestation. Third, traditional scoring systems do not allow charting individuals' change over time in the face of changing behavioral manifestation. Fourth, assessments are not well-suited for capturing individual differences, despite the dimensional nature of psychopathology. We propose solutions to these challenges by introducing a developmental scaling framework for psychopathology. Using externalizing psychopathology as an example, we draw on our content analysis of externalizing measures, as well as lessons from the study of other constructs across lengthy developmental periods. Addressing these challenges will allow charting individuals' trajectories of psychopathology across the lifespan and identifying risk and protective factors that influence developmental courses.

Keywords: psychopathology, lifespan development, longitudinal trajectories, heterotypic continuity, externalizing behavior problems

Studying Development of Psychopathology Across the Lifespan

NOTE: this manuscript is in progress of being drafted—it is incomplete.

When entrenched, psychopathology is costly and difficult to treat (Insel, 2008; Wakschlag et al., 2019). Understanding its developmental origins is therefore crucial for preventing later, more severe psychopathology. Developmental science seeks to build a bridge that spans the lifespan—from infancy to older adulthood—and not just transitory periods in people's lives. However, the field has not adequately studied individuals' development of psychopathology across the lifespan, instead taking a piecewise approach to development that focuses on narrower age ranges. The focus on narrow age ranges restricts our ability to understand development across important transitions, such as puberty, school entry, and entry into parenthood, thus limiting our understanding of lifespan development and the origins of psychopathology. In this paper, we describe reasons why this is the case—beyond logistical challenges of time and money. And we propose solutions to these challenges by introducing a developmental scaling framework for psychopathology. Using externalizing psychopathology as an example, we draw on our content analysis of over 270 externalizing measures, as well as lessons from the study of other constructs such as academic and cognitive skills across lengthy developmental periods.

The Goal

A key goal of developmental science is to understand how people develop across the lifespan. A variety of methods, including cross-sectional and longitudinal designs, could be leveraged for this purpose. In a cross-sectional design, people are assessed at one timepoint. In a longitudinal design, the same individuals are followed across time.

Cross-sectional designs provide the ability to cheaply and quickly assess a wide age range, providing a snapshot at a particular point in time. Cross-sectional designs may inform our understanding of general normative patterns of change—such as the typical pattern of cognitive growth and decline across the lifespan.

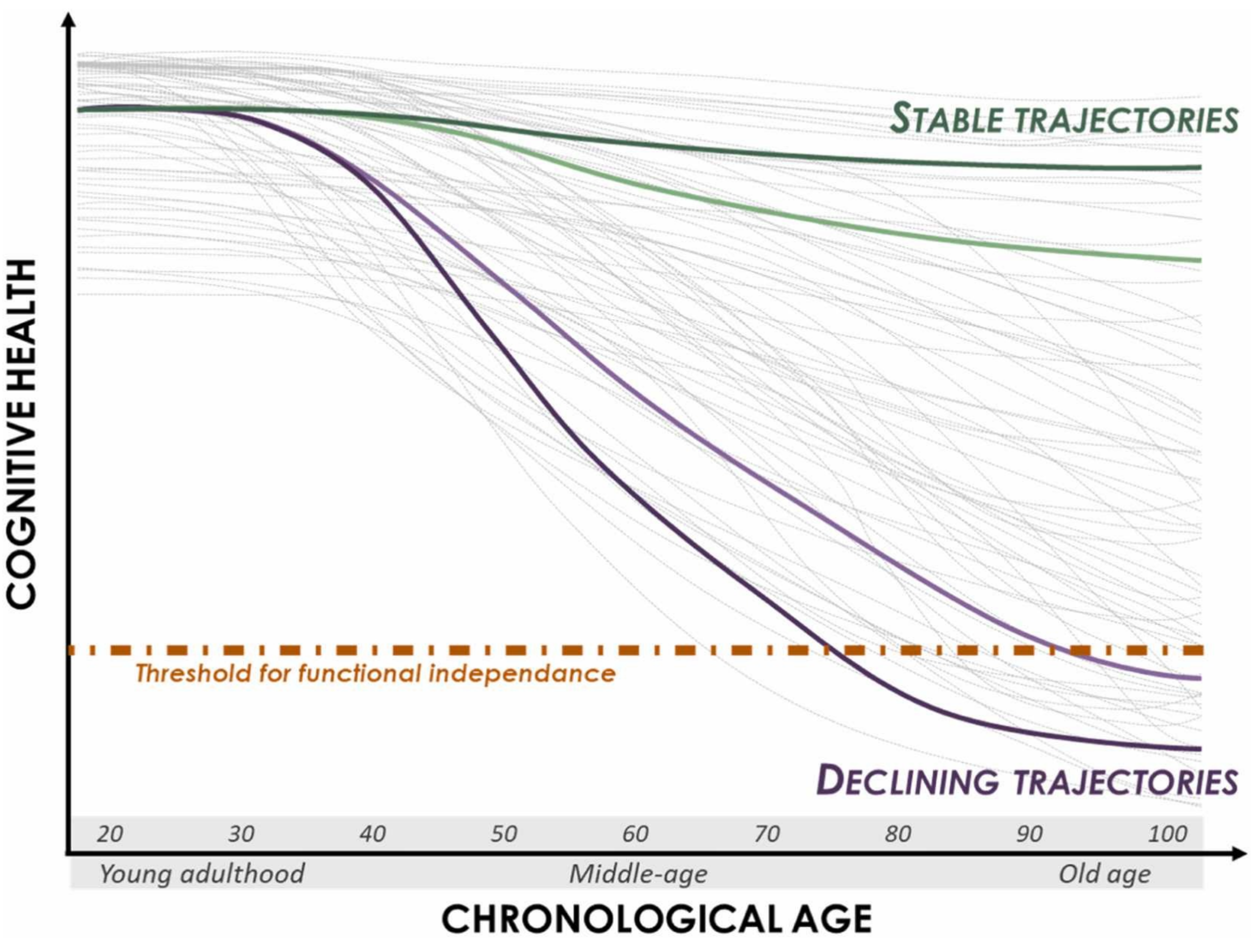
However, cross-sectional designs have key limitations. First, cross-sectional designs do not allow examining individuals' change over time. Second, cross-sectional designs may obscure diverse patterns of change. Individuals show considerable heterogeneity in level and change across time in many constructs. As an example, despite the normative pattern of steep cognitive decline in episodic memory with age, there are a subset of individuals who do not show this same pattern. For instance, superagers are individuals who are older adults (age 80+) who have episodic memory at or above the normative level of middle-aged adults (50–65 years old) and who may be resilient to the typical aging process (Godoy et al., 2021). A theoretical depiction of the heterogeneity of cognitive decline is depicted in Figure 1.

Third, in a cross-sectional study, any apparent age-related differences in a construct could reflect cohort differences instead of developmental changes. For example, average differences in stress between 5-year-olds and 10-year-olds could reflect the differing experiences that the 10-year-olds shared—such as going to school during the Coronavirus pandemic. Fourth, findings from cross-sectional studies can differ from findings examining the same people over time, which violates the convergence assumption. In cross-sectional studies, the convergence assumption is the assumption that cross-sectional age differences and longitudinal age changes converge onto a common trajectory. An example where the convergence assumption has been shown to be violated is in studies of cognitive decline. In particular, cross-sectional studies overestimate age-related cognitive declines compared to following the same people over time in a longitudinal study, which may reflect cohort effects such as the Flynn effect (where population intelligence scores at a given age tend to increase across generations; Ackerman, 2013).

Because longitudinal designs follow the same person(s) over time, such a design allows examining individuals' change across time. However, as we describe later, longitudinal designs also have potential confounds of change. Moreover, compared to cross-sectional designs, longitudinal designs take more time and tend to be more costly and to require more personnel. Thus, longitudinal studies that span the lifespan are not common. However, even cross-sectional work spanning the lifespan is not often conducted.

Figure 1

Theoretical illustration of inter-individual variability in cognitive aging trajectories. Each grey line represents one individual hypothetical cognitive health trajectory. Established and emerging mechanisms that determine whether a given individual experiences more stable (in green) vs. declining (in purple) trajectories are the focus of this review. The orange line marks the threshold for functional independence; once an individual's cognitive health drops below this level, autonomous daily living is compromised. (Figure reprinted from Abrous et al. (2026), Figure 1, p. 2. Abrous, D. N., Blin, N., Boraxbekk, C.-J., Catheline, G., Fitzsimons, C. P., Hilscher, M., Lemoine, M., Lopes, L. V., Maass, A., Nilsson, M., Nyberg, L., Rasmussen, L. J., Remondes, M., Sauvage, M., Schreiber, S., & Wolbers, T. (2026). Hallmarks of healthy cognitive aging: Inter-individual differences in aging trajectories. Ageing Research Reviews*, 119, 103102. https://doi.org/10.1016/j.arr.2026.103102)*



The Problem/Obstacle

Reasons for the Problem

Beyond logistical challenges of time and money, there are several key reasons why the field has failed to study people's development of psychopathology, longitudinally, across the lifespan. First, diagnostic and taxonomic systems do not define psychopathology constructs from a developmental perspective. A developmental perspective is crucial because psychopathology constructs often have earlier origins than their full clinical presentation, and forms of psychopathology frequently show changes in behavioral manifestation across development (heterotypic continuity). Second, current assessments do not adequately capture these changes in behavioral manifestation. Third, traditional scoring systems do not allow charting individuals' change over time in the face of changing behavioral manifestation. Fourth, current assessments are not well-suited for capturing the full range of individual differences, despite the dimensional nature of psychopathology. Fifth, as a result of these limitations, current assessments are not well-suited for capturing individuals' change over time.

Below, we describe these reasons in detail. To ground the discussion, we provide examples relating to externalizing psychopathology; however, these issues are not specific to externalizing.

Problems of our Constructs

The first reason the field has failed to adequately study individuals' development of psychopathology across the lifespan represents a theory problem—namely, problems with our constructs, and the codifying of those constructs in diagnostic and taxonimc systems. In particular, our theories do not fully specify how constructs manifest behaviorally across the lifespan. Moreover, where such theories do indicate changes in behavioral manifestation across development, diagnostic and taxonomic systems do not incorporate these developmental changes.

Lack of a Developmental Perspective

Diagnostic and taxonomic systems do not define psychopathology constructs from a developmental perspective. The diagnostic systems, including the Diagnostic and Statistical

Manual of Mental Disorders (DSM-5-TR; American Psychiatric Association, 2022) and the International Classification of Diseases (ICD-11; World Health Organization, 2022), specify various externalizing-related disorders, including attention-deficit/hyperactivity disorder (ADHD), oppositional defiant disorder (ODD), conduct disorder (CD), and antisocial personality disorder (ASPD). Among externalizing-related disorders—with few exceptions[1]—the diagnostic criteria do not differ across development, even though diagnostic manuals acknowledge that developmental factors impact symptom presentation[2].

Problems of Existing Assessments and Traditional Scoring Systems

Other key reasons the field has failed to adequately study individuals' development of psychopathology across the lifespan represent method problems—namely, problems with existing

[1] In the DSM-5-TR (American Psychiatric Association, 2022), the minimum frequency for considering a symptom of ODD to be present is "most days" (for a period of six months) for children younger than 5 years, whereas it is "at least once per week" (for at least six months) for children 5 years or older. For the diagnosis of ADHD, six or more symptoms are required for children and younger adolescents (up to age 16), whereas five or more sympomts are required for older adolescents and adults (aged 17 or over). However, the symptoms for defining ODD and ADHD do not differ across development.

[2] In the DSM-5-TR (American Psychiatric Association, 2022), some disorders have a "Development and Course" subsection that describes when disorders tend to emerge and their rates across ages, in addition to describing the types of symptoms that may be more or less common at various points in development. For instance, in the section on major depressive disorder, it is noted that "hypersomnia and hyperphagia are more likely in younger individuals, and melancholic symptoms, particularly psychomotor disturbances, are more common in older individuals. Depressions with earlier ages at onset are more familial and more likely to involve personality disturbances." (p. 189). In the section on ADHD, it is noted that, "In preschool, the main manifestation is hyperactivity. Inattention becomes more prominent during elementary school. During adolescence, signs of hyperactivity (e.g., running and climbing) are less common and may be confined to fidgetiness or an inner feeling of jitteriness, restlessness, or impatience. In adulthood, along with inattention and restlessness, impulsivity may remain problematic even when hyperactivity has diminished." (p. 71). In the section on CD, it is noted that "Symptoms of the disorder vary with age as the individual develops increased physical strength, cognitive abilities, and sexual maturity. Symptom behaviors that emerge first tend to be less serious (e.g., lying, shoplifting), whereas conduct problems that emerge last tend to be more severe (e.g., rape, theft while confronting a victim)… When individuals with conduct disorder reach adulthood, symptoms of aggression, property destruction, deceitfulness, and rule violation, including violence against co-workers, partners, and children, may be exhibited in the workplace and the home" (p. 534). Moreover, some disorders in the DSM-5-TR note that, to be considered a symptom, the frequency, intensity, or persistence, of the behavior must be inconsistent with the level expected for their developmental level. For instance, in the diagnosis of ADHD, it is noted that "the symptoms have persisted for at least 6 months to a degree that is inconsistent with developmental level" (p. 68). In the diagnosis of ODD, it is noted that "other factors [besides a minimum frequency] should also be considered, such as whether the frequency and intensity of the behaviors are outside a range that is normative for the individual's developmental level, gender, and culture." (p. 522). In addition, some disorders require that some symptoms onset before or after a particular age. For instance, in the diagnosis of ADHD, several symptomers must be present prior to age 12. In the diagnosis of CD, the symptoms "often stays out at night despite parental prohibitions" and "if often truant from school" must begin before age 13 (p. 531). In the diagnosis of ASPD, symptoms must be persistent since age 15 but have onset prior to age 15.

assessments and how those assessments are used, based on traditional scoring systems. There is a mismatch metween theory and method. In particular, there is a mismatch between theory of constructs—including how a construct manifest behaviorally—and our methods to assess the construct and to evaluate people's level and change in that construct.

Do Not Adequately Capture Constructs' Change in Behavioral Manifestation Across Development

Do Not Allow Charting Individuals' Change Over Time When the Construct Changes in Behavioral Manifestation

Not Well-Suited For Capturing the Full Range of Individual Differences

Not Well-Suited for Capturing Individuals' Change Over Time

Example: Externalizing Psychopathology

Solution: A Developmental Scaling Framework

Other Key Challenges to Address

Potential Confounds of Change

Practice/Retest Effects

Period and Cohort Effects

Measurement Error

Change in Meaning of the Scores

Factorial Invariance Across Time

Obsolescence of Items and Measures

Conclusion

Addressing these challenges will allow charting individuals' trajectories of psychopathology across the lifespan and identifying risk and protective factors that influence their developmental courses. Doing so is critical for understanding how psychopathology develops, when to intervene, and how to most effectively prevent psychopathology. Although this manuscript focuses on development of psychopathology, the principles discussed likely apply to many other developmental domains.

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Appendix A

This Section Is an Appendix

Appendix B

Another Appendix